



U.S. Department
of Transportation

Memorandum

National Highway Traffic Safety Administration

Date: March 11, 2026

From: Mole Clary
Contracting Officer's Representative
Office Of Emergency Medical Services, NPD-400

To: Gregory Benjamin II
Contracting Officer
Office of Acquisition Management (OAM), NFO-300

Subject: Approval of Justification for Other Than Full and Open Competition (JOTFOC) for a new contract titled, *National EMS Information System Technical Assistance Center*, with University Of Utah.

This document serves as Justification for Other Than Full and Open Competition to establish a bridge contract for a period of performance of one (1) year. This will permit continuation and coordination support services for Office Of Emergency Medical Services.

The Request For Proposal (RFP) for the new coordination support services contract, along with all required supporting documents, were submitted to OAM in December 2024; however, it has not yet cleared small business review, and the current extension contract is set to expire on March 31 2026.

U.S. Department of Transportation
National Highway Traffic Safety Administration
Justification for Other Than Full and Open Competition
National EMS Information System Technical Assistance Center
Period of Performance: 4/1/2026 – 3/31/2027

IDENTIFICATION OF AGENCY

National Highway Traffic Safety Administration (NHTSA)
1200 New Jersey Avenue, SE, Washington, DC 20590

Office of Acquisition Management (OAM) - NFO-300

Gregory Benjamin II
Contracting Officer

Office of Emergency Medical Services (EMS) - NPD-400

Clary Mole
Contracting Officer Representative, Level III

Jeremiah Kinsman
Project Manager

Office of Info Management – NIO-200

Joyce Padela
Alternate Contracting Officer Representative, Level III

BACKGROUND

The mission of the National Highway Traffic Safety Administration (NHTSA) is to save lives, prevent injuries, and reduce economic costs due to motor vehicle crashes through education, research, safety standards, and enforcement. The Highway Safety Act of 1970 established NHTSA and tasked it with implementing safety programs under the National Traffic and Motor Vehicle Safety Act of 1966 and the Highway Safety Act of 1966.

NHTSA's Office of EMS conducts various research and development activities, including demonstration projects, training, education and data collection relating to emergency medical services, including the transportation of the injured to reduce death and disability resulting from motor vehicle crashes (MVCs). Pursuant to 23 U.S.C. § 401 et seq., EMS supports programs that further develop EMS infrastructure and the delivery of pre-hospital, post-crash care. This acquisition enables the continued operation of National EMS Information System (NEMSIS), the National EMS Database (NEMSD), and the Technical Assistance Center (TAC), which together capture EMS records, improve data quality, support system evaluation, and promote integration with traffic safety, emergency response, and health information systems.

Highway safety outcomes depend in part on the rapid and effective response of EMS following a crash. However, for many years, the EMS community lacked access to standardized national data. NHTSA and its partners addressed this gap through the creation of the NEMSIS, which provides a national standard for documenting out-of-hospital care and a technical infrastructure to collect and analyze EMS data at local, state, and national levels. This standardized data infrastructure empowers states and agencies to do their own system performance measurement, benchmarking, evaluation of outcomes, and focused improvement efforts. The TAC aids states and EMS agencies without staffing or capabilities to do their

own system performance assessments and outcome evaluations to develop guidance to specifically drive their systems improvements efforts. Examples of such assessments include (but are not limited to):

- Timeliness of response to motor vehicle crashes and timeliness of patient extrication from crashed vehicles;
- Appropriateness of resources dispatched to the scene of the crash;
- Effectiveness in timely treatment of life-threatening injuries and conditions (e.g. severe bleeding and traumatic brain injury);
- Effectiveness in application of evidence-based care for crash patients; and
- Effectiveness in making decisions about the transportation to the most appropriate hospital or trauma center.

NEMSIS captures and standardizes more than 60 million EMS activations annually. These data include detailed information on the timing, location, cause, and clinical response to medical and trauma-related emergencies including MVCs, a leading cause of death and serious injury in the United States. Records originate from EMS clinicians at the point of care and are transmitted from local EMS agencies through state, territorial, tribal, and District of Columbia (STTDC) data repositories to the NEMSD.

The NEMSIS data standard defines hundreds of data elements and associated values to ensure consistent, interoperable reporting across jurisdictions and software platforms. The system is critical to understanding pre-hospital care trends and patient outcomes for injuries and fatalities. Data quality is safeguarded through structured compliance testing of electronic patient care report (ePCR) software, adoption of national case definitions and schema, and robust validation at multiple levels.

The TAC plays a central role in maintaining this national data collection and infrastructure. It provides comprehensive education and training to EMS agencies, STTDC EMS offices, and software vendors; develops and maintains data submission and validation tools; administers data quality processes and compliance testing; produces standardized reports and visualizations; and supports analyses that inform national policies, system improvement efforts, and crash and injury surveillance.

The scope of responsibilities has significantly expanded in recent years to reflect the maturity and national significance of the NEMSIS program. Core services now include:

- Cloud and Infrastructure: Hosting within a Federal Information Security Management Act (FISMA) Moderate-level Amazon Web Services (AWS) environment with 24/7/365 availability, failover capabilities, system replication, and role-based access control.
- Cybersecurity: Strict adherence to DOT, NHTSA, Continuous Diagnostics and Mitigation (CDM), and FISMA policies, including encryption of data in transit and at rest, authentication via Personal Identity Verification (PIV) credentials, and public trust security clearances for personnel.
- Software and Data Tools: Support for an Online Analytical Processing (OLAP)-type data cube, dozens of role-specific dashboards, real-time data quality monitoring, and analytic feeds for advanced analysis through Environmental Data Management and Storage (EDMAS).
- Governance and Documentation: Robust version control, change tracking, cybersecurity scanning and reporting, configuration management, and compliance documentation.
- Data Use Agreements: Negotiate, execute, and maintain DUAs with STTDC partners.
- Interoperability and FHIR: A forthcoming migration from XML to Fast Healthcare Interoperability Resources (FHIR) standards, in alignment with federal mandates under the 21st Century Cures Act, requiring extensive planning, technical assistance, and infrastructure upgrades.

NEMSIS also continues to provide compliance checks, data validators, vendor onboarding tools, and interactive ports that need continuous immediate processing, real-time interaction, and scalable and reliable data ingestion. The expanding mission reflects NEMSIS's evolution into a mature federal data platform with increasing demands for technical stewardship, cybersecurity, real-time data services, and cross-sector interoperability.

DESCRIPTION OF SERVICE REQUIREMENT OF AGENCY

NHTSA requires the continued operation and implementation of NEMSIS, the NEMSD, and the TAC. The Contractor will operate the TAC, hosting center, NEMSD, website (www.NEMSIS.org), and all related functions. Related functions include, but are not limited to, maintaining a secure, high-availability data infrastructure; ensuring compliance with federal information technology (IT) and cybersecurity standards; advancing national EMS data standards; and supporting integration with traffic safety, public health, and healthcare information systems. The Contractor **MUST** have demonstrated extensive experience working with the EMS data at the local (agency), state and federal levels.

The Contractor will implement all approved architecture, structure, and artifacts associated with the NEMSIS 3.5.0 and 3.5.1 data standards and all future data standard releases. The Contractor must demonstrate capabilities in system administration, cloud computing, data analytics, EMS stakeholder coordination, data governance, data visualization, ePCR software compliance testing and technical assistance. The services will include oversight of national EMS data submission, collection, validation, reporting, interoperability, and research use. This work directly supports national highway safety goals by enabling data-driven improvements in EMS response, post-crash care, and health outcomes. The Contractor is expected to apply a technically innovative, reliable, and standards-based approach to maintain the integrity, scalability, and usability of NEMSIS data infrastructure.

The Contractor will operate all related systems to ensure uninterrupted collection, management, and reporting of EMS data received from STTDC EMS agencies. Currently NHTSA's OEMS has three Interagency Agreements (IAAs) for the use of NEMSIS data, the TAC and analytic services. In addition, many federal, state, and local EMS agencies are provided analytic and support services at no cost if the request is within the current scope of work.

The Contractor will provide support for additional tasks designed to facilitate data connectivity and stakeholder utilization of the NEMSIS standard data. This will be accomplished in tandem with maintenance of an AWS Cloud (FISMA Moderate) NEMSIS IT infrastructure instance. The NEMSIS shall be available 24 hours a day, 7 days a week, 365 days a year unless a scheduled maintenance period or mitigation of an unexpected event is required, uptime of 99.9% shall be maintained throughout the period of performance. The government also requires personnel with specialized healthcare backgrounds.

DESCRIPTION OF ACTION TO BE APPROVED

NHTSA proposes to award a sole-source, 1-year (12 months) bridge contract to the **University of Utah** to continue operating the **National EMS Information System (NEMSIS) Technical Assistance Center (TAC)**. The estimated value is \$8,000,000.00 for a short-term continuation of essential IT services while the Government completes market research and a full and open competition for a new, long-term NEMSIS TAC contract.

The NEMSIS TAC provides nationally critical IT and programmatic services, including:

- Management and maintenance of the NEMSIS data standard;
- Operation of national EMS data submission, validation, storage, and analytics environments;

- Technical assistance to States, Territories, and EMS software vendors;
- Data quality monitoring, system interoperability support, and schema governance; and
- Public data reporting, dashboards, and analytical tools supporting EMS system improvement.

These services ensure continuous functioning of the nation's standardized EMS data infrastructure, required by all 56 States and Territories and relied upon by national program offices, researchers, policymakers, and system developers.

STATUTORY AUTHORITY FOR OTHER THAN FULL AND OPEN COMPETITION

This acquisition is conducted under U.S.C. 3304(a)(1) and RFO Part 6 Model Deviation Text FAR 6.103-1, *Only One Responsible Source and No Other Supplies or Services Will Satisfy Agency Requirements*, for a follow-on bridge requirement where competition is not feasible without unacceptable risk to mission continuity.

JUSTIFICATION FOR EXCEPTION

NHTSA is currently preparing to procure information technology services for a period of performance of 10 years and at an approximate cost of \$80M for the future NEMSIS TAC contract. While the Government completes a FAR compliant and efficient procurement process, a bridge period is necessary to support NEMSIS TAC continuity. This bridge contract must be a sole-source award because the University of Utah is the **only responsible source** capable of performing uninterrupted operations of the NEMSIS TAC. Key factors include:

A. Continuity of Critical National EMS Data Operations

The TAC manages real-time collection, validation, and storage of millions of EMS patient care records submitted by States and Territories. Any disruption would immediately impact:

- State EMS reporting and regulatory obligations;
- National injury and public health surveillance;
- EMS software vendors reliant on NEMSIS validation services; and
- Federal programs that rely on national EMS data for research, analytics, and policy.

Transitioning to a new vendor requires extensive technical onboarding, system knowledge transfer, codebase review, and re-establishment of secure interfaces. Attempting such a transition during this short interim would risk data integrity and availability, as well as a lapse in critical public safety services. If a lapse were to occur, relaunching NEMSIS TAC would create significant costs and be an inefficient use of agency and vendor time and resources.

B. Technical Knowledge and Complex System Integration

The University of Utah currently maintains and operates:

- The NEMSIS data repositories and schema management environment;
- The nationally standardized validation suite used by all EMS software vendors;
- Custom analytics tools, collection systems, and application programming interfaces (APIs); and
- Documentation, metadata, and configuration assets not yet fully prepared for transfer.

The University of Utah maintains a 485-million record database that requires specific and secure technologies. To transfer all existing artifacts, systems architectures, documentation, and operational databases would require significant time, resources, and training. Further, the University of Utah would be tasked with training a new custodian on a decade of technical system knowledge.

For example, the database is protected by complex Data Use Agreements (DUAs) and rigorous security protocols. Every state and territory have a custom DUA with the University of Utah. Transferring these legal frameworks to a new entity requires individual state attorney general approvals—a process that historically takes years, not months. Without these agreements in place on Day 1, a new custodian would legally be unable to receive or process data from most states.

In addition, the NEMSIS TAC actively governs the Schematron validation rules that every local ePCR software uses to function. A new custodian would inherit a library of over 450 active edit checks and business rules that must be perfectly synchronized across hundreds of software vendors. If a single "Critical Patch" update (such as the recent Version 3.5.0 CP6) is mismanaged, local agencies could see a 100% rejection rate for their patient records overnight.

The critical steps required to ensure an efficient transition to a new custodian, is not feasible without an unacceptable risk of prolonged system downtime, data loss, and the immediate halting of critical national EMS data collection and compliance testing.

C. Risk of Service Interruption Without a Bridge Contract

Failure to award a short-term bridge contract would result in immediate loss of:

- National EMS data validation and collection
- Technical assistance to States and vendors;
- Governance over the NEMSIS standard; and
- Required reporting tools relied on by Federal and State partners.

University of Utah is currently the only entity capable of performing uninterrupted operations of these services. No other vendor possesses the required operational access or system knowledge to assume control of these services without a deliberate transition period, which would cause a disruption to the continuous collection of over 60 million annual EMS records.

The absence of national data collection would immediately halt critical Federal and public health surveillance tools, including the drug overdose dashboard funded and relied upon by the White House Office of National Drug Control Policy. A disruption in the administration of the NEMSIS TAC contract would have significant negative impacts to current EMS data infrastructure. Because the TAC is the central "brain" for the national EMS data standard, local agencies would lose the technical support and validation systems required to keep their operations running smoothly.

Ultimately, this lapse would weaken the national EMS data ecosystem and degrade the Government's ability to drive improvements in public health, emergency response, and post-crash care while a new environment is stood up.

MARKET RESEARCH

NHTSA is currently preparing for the future NEMSIS TAC contract, and preliminary market research for that procurement indicates that multiple vendors may be capable of competing for the new 10-year contract. However, none of these vendors, other than the University of Utah, are **immediately** capable of continuing operations without an unacceptable mission risk.

PERIOD OF PERFORMANCE

The proposed bridge contract will cover **12 months**, with no additional option periods, aligning with the timeline required to complete the procurement process and transition to the follow-on awardee.

DETERMINATION OF PRICE FAIR AND REASONABLENESS

The Contracting Officer (CO) finds that the anticipated cost to the Government for this bridge contract will be fair and reasonable. This determination is not based on a single data point but rather a holistic evaluation of the market, historical data, and the specific requirements of the solicitation. The \$8,000,000 estimate was derived from a comparison of the previous year's actuals (\$7.6M) adjusted by a 5.2% factor to account for increased data storage requirements and updated Department of Labor Wage Determinations. This methodology ensures the government maintains critical EMS data continuity at a price consistent with historical market performance.

Price reasonableness will be ensured by comparing proposed labor rates and indirect costs directly against the established rates in the predecessor contract (DTNH2215C00030). The Government will evaluate all proposed price increases to ensure they are consistent with current market trends, and an updated Independent Government Cost Estimate (IGCE) will be utilized that reflects the short-term nature of this bridge period.

A technical evaluation will be preformed to verify that the labor mix remains consistent with the successfully performed services under the previous period of performance. Based on these planned analysis techniques, the final negotiated price is expected to be fair, reasonable, and represent the best value to the Government.

SIGNATURE PAGE

CERTIFICATION – REQUIRING ACTIVITY/PROGRAM OFFICE

In accordance with FAR 6.302-1, the supporting data which form the basis for this justification have been certified as complete and accurate by the technical personnel:

CLARY DENNIS
MOLE JR

Digitally signed by CLARY
DENNIS MOLE JR
Date: 2026.03.11 15:37:58
-04'00'

Clary Mole
Contracting Officer's Representative
Office Of Emergency Medical Services

Date

CERTIFICATION – CONTRACTING OFFICER

As confirmed by the signature below, the Contracting Officer hereby certifies that the facts and representations included in this justification are accurate and complete to the best of her knowledge and belief.

In accordance with **FAR 15.404-1**, the Contracting Officer has analyzed the proposed costs for the subject requirement, and based on the application of [Insert Method, e.g., Adequate Price Competition / Comparison to Historical Prices], has determined that the anticipated costs are **fair and reasonable**.

Based on the foregoing justification, the use of Other than Full and Open Competition procedures is hereby considered appropriate for the proposed contract to the University of Utah School of Medicine, pursuant to the authority of FAR 6.302-1.

GREGORY BERNARD
BENJAMIN II

Digitally signed by GREGORY
BERNARD BENJAMIN II
Date: 2026.03.11 15:00:55
-05'00'

Gregory Benjamin, II
Contracting Officer
Office of Acquisition Management

Date

CERTIFICATION – ADVOCATE FOR COMPETITION

Based on the foregoing justification, the use of Other than Full and Open Competition procedures is hereby considered appropriate for the proposed contract to the University of Utah School of Medicine pursuant to the authority of FAR 6.302-1.

SEAN METRICE
CLAYTON

Digitally signed by SEAN
METRICE CLAYTON
Date: 2026.03.13 15:12:32 -04'00'

Sean Clayton
NHTSA Competition Advocate

Date